

Prevention and Management of Pressure Ulcers Policy

Issue Date	Review Date	Version
November 2024	November 2029	5

Purpose

This policy will enable compliance with current guidelines and recommendations for prevention and management of pressure ulcers, including National Wound Care strategy (NWCS) and NICE guidance. Effective implementation will reduce co-morbidity, enables patients to return their normal activity and home or place of care, reduce inpatient time and health care costs. The application will improve communication and ensure consistency of care across healthcare providers.

Who should read this document?

All healthcare workers, including healthcare assistants, providing care to inpatients, in all care groups.

Key Messages

Pressure ulcers are largely avoidable if action is taken to prevent and manage patients appropriately and at an early stage.

The following actions are needed:

- Assessment of risk
- Assessment of the skin condition
- Planning patient care based on assessment findings
- Implementation of prevention strategies with appropriate documentation
- Evaluation of patient response to risk prevention and needs management
- Consideration of safeguarding
- Communication with the patient, their family and/or carers and the ward team about pressure ulcer prevention and management

Core accountabilities

Owner	Tissue Viability Lead Nurse
Review	Harm Free Team
Ratification	Chief Nurse
Dissemination (Raising Awareness)	Tissue Viability Lead Nurse
Compliance	Ward Managers and Matrons

Links to other policies and procedures

Wound Care Policy
 End of Life Care in Hospital Policy
 Incident Management Policy
 Patient Safety Incident Response Plan
 Moving and Handling People and Objects Policy
 Nutrition and Hydration Policy
 Appropriate Prescribing for Oral Nutritional Supplements (ONS) Policy
 The Management and Use of Medical Devices Policy
 Medical Devices User Guide
 The Safeguarding Adults at Risk Policy
 Child Protection Policy
 Carers Policy
 Purpose T, Skin Assessment
 Intentional Care Record

Version History

1	May 2010	Initial document
2	August 2012	Revised and reformatted
3	July 2019	Revised
4	December 2020	Revised
5	November 2024	Reviewed and Updated

Equality, diversity, and inclusion is at the heart of the NHS strategy and for UHP this means ensuring we create an equitable organisation for colleagues, patients, and visitors. Our inclusion agenda recognises that everyone matters, as well as understanding that there are times when people, particularly those from protected groups (defined by the Equality Act 2010 - age; disability; gender reassignment; marriage and civil partnership; pregnancy and maternity; race; religion or belief; sex; sexual orientation), may face unfairness and discrimination.

We have a moral responsibility to work in a way that creates fairness as well as a legal and public sector equality duty which we take very seriously. We will always aim to create policies and practices that eliminate workplace discrimination and health inequalities, as well as strengthening our relationships within the community and with our colleagues who may identify with one or more protected groups.

A diverse workforce improves the experience of our patients and creates an environment where everyone belongs and feels included. UHP recognise that we all play a part in providing exceptional care to patients as well as showing kindness and compassion to patients and colleagues.

An electronic version of this document is available in the Document Library. Larger text, Braille and Audio versions can be made available upon request.

Contents

Section	Description	Page
1	Introduction	4
2	Purpose	4
3	Definitions	4
4	Duties	6
5	Standards- Key Performance Indicators	6
6	Overall Responsibility for the Document	11
7	Consultation and Ratification	11
8	Dissemination and Implementation	12
9	Monitoring Compliance and Effectiveness	12
10	References and Associated Documentation	13
Appendix 1	Dissemination Plan	14
Appendix 2	Equalities and Human Rights Impact Assessments	16

1 Introduction

- 1.1. Pressure ulcers can cause significant pain and distress for patients and can contribute to longer stays in hospital, increasing the risk of complications, including infection and they also cost the NHS in the region of more than £1.4 million every day (Guest et al. 2017). Many pressure ulcers are avoidable if basic knowledge is provided and preventative best practice is followed.
- 1.2. University Hospitals Plymouth NHS Trust (UHP) recognises that the prevention and management of pressure ulcers are crucial to good patient outcomes and improving the patient experience.

2 Purpose

- 2.1. The Trust will ensure that all patients admitted to hospital for a period of 24 hours or more receive a pressure ulcer risk assessment alongside a skin assessment within 6 hours of their arrival to hospital. All appropriate measures will be taken to prevent the development of pressure ulcers while in the care of the Trust.
- 2.2. Where patients are admitted with pressure ulcers or develop them in our care, treatment will be administered that prevents any further deterioration and promotes healing.
- 2.3. This policy is intended to establish a standardised approach and framework for healthcare professionals undertaking the care of patients (all ages) with a pressure ulcer, or at risk of developing one.
- 2.4. The policy and guidelines are based on current best practice statements, position documents, expert opinion, national and European guidelines and research evidence where it exists.
- 2.5. The development of pressure ulcers may in some instances be an indication of neglect by the care provider (whether on admission or during inpatient stay). If there is a concern that an adult at risk may have been abused or is at risk of abuse, a safeguarding adults referral must be made. It must also be recognised that some pressure ulcers may not be preventable.

3 Definitions

- 3.1. A pressure ulcer is a localised damage to the skin and/or underlying tissue, usually over a bony prominence (or related to a medical or other devices), resulting from sustained pressure (including pressure associated with shear). The damage can be present as intact skin or an open ulcer and may be painful.
- 3.2. Categorisation guidance below adapted from: NHS Improvement Pressure ulcer categorisation group (2019) Pressure Ulcer Categorisation. UHP categorisation poster available on Health Toolbox.

Category 1—Non-blanchable Erythema	Intact skin with non-blanchable redness of a localised area, usually over a bony prominence. Darkly pigmented skin may not have visible blanching; its colour may differ from the surrounding area. The area may be painful, firm, soft, warmer or cooler compared to adjacent tissue. Category 1 may be difficult to detect in individuals with dark skin tones. May indicate 'at risk' individuals.
Category 2—Partial Thickness Skin Loss	Partial thickness loss of dermis presenting as a shallow open ulcer with a red pink wound bed, without slough. May also present as an intact or open/ruptured serum-filled blister. Presents as a shiny or dry shallow ulcer without slough or bruising. This category should not be used to describe skin tears, tape burns, perineal dermatitis, maceration or excoriation.
Category 3—Full Thickness Skin Loss	Full thickness tissue loss. Subcutaneous fat may be visible, but bone, tendon or muscle are not exposed. Slough may be present but does not obscure the depth of tissue loss. May include undermining and tunnelling. The depth of a Category 3 pressure ulcer varies by anatomical location. The bridge of the nose, ear, occiput and malleolus do not have subcutaneous tissue, and Category 3 ulcers can be shallow. In contrast, areas of significant adiposity can develop extremely deep Category 3 pressure ulcers. Bone/tendon is not visible or directly palpable.
Category 4: Full thickness tissue loss	Full thickness tissue loss with exposed bone, tendon or muscle. Slough or eschar may be present on some parts of the wound bed. Often includes undermining and tunnelling. The depth of a Category 4 pressure ulcer varies by anatomical location. The bridge of the nose, ear, occiput and malleolus do not have subcutaneous tissue, and these ulcers can be shallow. Category 4 ulcers can extend into muscle and/or supporting structures (eg fascia, tendon, joint capsule) making osteomyelitis possible. Exposed bone/tendon is visible or directly palpable.
Unstageable—minimum category 3: depth unknown but there will be full thickness skin loss	Full thickness tissue loss in which the base of the ulcer is covered by slough (yellow, tan, grey, green or brown) and/or eschar (tan, brown or black) in the wound bed. Until enough slough and/or eschar is removed to expose the base of the wound, the true depth cannot be determined. It will be a minimum of category 3, but may reveal a category 4 ulcer.
Suspected deep tissue injury (DTI): depth unknown	Purple or maroon localised area of discoloured intact skin or blood-filled blister due to damage of underlying soft tissue from pressure and/or shear. The area may be preceded by tissue that is painful, firm, mushy, boggy, warmer or cooler compared to adjacent tissue. Deep tissue injury (DTI) may be difficult to detect in individuals with darker skin tones. Progression may include a thin blister over a dark wound bed. The wound may further develop and become covered by thin eschar. Development may be rapid, exposing additional layers of tissue even with optimal treatment.

- 3.3 .Device Related Pressure Ulcer. A pressure ulcer that forms as a direct result of pressure/friction/shear caused by a medical device. With the exception of mucosal pressure ulcers, these should be 'categorised' as above and the device causing the pressure noted.
- 3.4 .Moisture Associated Skin Damage (MASD). Damage to the skin that occurs as a direct result of excess levels of moisture at the skin surface (e.g. urine, faeces, sweat, wound exudate).
- 3.5 .aSSKINg Bundle. A bundle of care that addresses Assessment, Skin assessment, Surface, Keep moving, Incontinence and Nutritional needs of the patient, Giving information.
- 3.6 Present on Admission (POA) Pressure Ulcer. A pressure ulcer that is observed during the skin assessment undertaken on admission to that service.
- 3.7 New (acquired) Pressure Ulcer. The definition of a new pressure ulcer within a setting is that it is first observed within the current episode of care.

4 Duties

- 4.1. All clinical staff are responsible for the initial and ongoing assessment of patients for the risk of acquiring pressure ulcers, escalating and reporting any incidence of pressure ulcers or any safeguarding concerns, and for the delivery of preventative measures and treatment of pressure ulcers.
- 4.2. All Ward/Departmental Managers are responsible for ensuring the policy is implemented by staff in their area of responsibility, to monitor and investigate any incidence of new hospital-acquired pressure ulcer and to implement an annual action plan with specific actions to reduce overall incidence/incidents.
- 4.3. All Matrons are responsible for ensuring the policy is implemented in the area of responsibility and providing assurance to the Care Group and Service Line Management Team of compliance with the policy.
- 4.4. Care Groups and Service Lines must review and monitor compliance with the policy. They must identify any areas of concern and implement changes to practice where required. A care group specific action plan is encouraged.
- 4.5. The Tissue Viability Team provides support for staff implementing the policy and specialist advice where required for patient management. Monitor the effectiveness of the policy and provide support for Matrons/Ward Managers to develop and implement action plans, report to the Nursing and Midwifery Operational Committee on the effectiveness of the policy.

5 Standards- Key Performance Indicators

5.1. On admission

- 5.1.1. All patients will receive an assessment of their risk of developing pressure ulcers and comprehensive skin assessment as part of the overall assessment within six hours of admission and following any transfer of care

i.e. ward to ward, theatre to ward. This includes admission via any route, such as ED, maternity or SAU.

- 5.1.2. The risk assessment and the findings from the assessment of the patient's skin must be recorded on the PURPOSE T risk assessment (or agreed modified version of) in the nursing admission notes/record.
- 5.1.3. A clear plan of care and any interventions must be documented and should consider the care guidance available on the risk assessment form as well as all elements of the aSSKINg bundle (HIA, 2009). If indicated, intentional care rounding and appropriate documentation should be commenced.
- 5.1.4. The site, extent and category of any skin damage should be documented and if wound dressings are required, a wound assessment chart must be used.
- 5.1.5. All Patients admitted with skin damage or acquiring it in hospital must have a clinical incident (Datix) form completed within 24 hours.
- 5.1.6. Consideration needs to be given to patients with skin damage about whether a safeguarding concern should be raised with the local authority.
- 5.1.7. All pressure ulcers should have a digital image taken, using SNAP SeeEHR unless the patient makes the decision not to consent. Care should be taken to maintain patient dignity.
- 5.1.8. Ward/Department Managers are responsible for ensuring the above actions are completed as required by their teams.

5.2. Prevention

- 5.2.1. All unit/ward nurses will develop a plan of care for all patients considered at risk of pressure damage (orange or red on PURPOSE T). This plan will include, pressure relieving equipment, positioning schedules, skin care requirements (including continence management), moving and handling needs, nutritional requirements and patient education and the provision of information (See NICE clinical guideline CG179 and NWCS Pressure Ulcer Recommendations and Clinical Pathway (2023) for further advice). Consideration needs to be given to all aspects of the aSSKINg care bundle and where necessary, ongoing use of intentional rounding.
- 5.2.2. The plan of care will take into consideration the findings of the risk assessment, patient comfort and preferences, critical care needs, patient weight (BMI) and MUST assessment, general health, skin assessment, continence issues and any safety issues.
- 5.2.3. If equipment required is not in place within 24 hours, a clear plan of care must be documented which includes actions taken to minimise the risk of damage (e.g. increased frequency of repositioning) and the plan for obtaining equipment.
- 5.2.4. Staff will seek specialist advice as appropriate, e.g. manual handling, dietician, tissue viability team. Referral will be documented in the nursing/medical notes. When the specialist advice has been provided the

nurse will ensure it is incorporated into the patient's plan of care within 24 hours.

5.2.5. A PURPOSE T risk assessment will be documented every 72 hours (or more frequently if the patient's clinical condition changes significantly) and the aSSKINg care bundle updated to reflect any changes.

5.2.6. New skin damage will be documented on the body map or in the patient records when identified.

5.3. Moisture Associated Skin Damage (MASD)

5.3.1. MASD, also called Incontinence Associated Dermatitis (IAD) or Nappy Associated Dermatitis (NAD) when caused by urine or faeces, can be very painful. It can be classified as mild, moderate and severe and should be reported via Datix when it occurs.

5.3.2. Although this is not caused by pressure, MASD puts the patient at increased risk of pressure ulceration. Therefore, it is important to manage signs of MASD early.

5.3.3. Staff will follow the IAD Clinical Pathway (NAD Pathway for children/babies) and use a 'step up, step down' approach to treatment. Cases of severe MASD/IAD/NAD or skin damage that is not resolving should be referred to the Tissue Viability Team via SeeEHR with details of the care already given.

5.3.4. Overuse of incontinence pads will be avoided and hygiene checks may need to be increased to prevent or manage symptoms. This will be planned within the aSSKINg care bundle.

5.4. Treatment

5.4.1. All patients with pressure damage must have a datix incident form completed within 24 hours of identification. A referral to tissue viability must be made for all patients with category 3, 4, or unstageable – minimum category 3 pressure ulcers, or deep tissue injury (DTI), whether newly identified or present on admission to hospital. For patients at Derriford Hospital, this will be done using the referral form on SeeEHR and a digital image of the wound uploaded. For patients at the Community Hospitals, this will be done by emailing Livewell Southwest Tissue Viability team with a completed referral form and digital image of the wound.

5.4.2. Once pressure damage has been identified it must be reassessed on each dressing change using the Trust categorisation tool (based on EPUAP and NWCS guidance); this should at the very least be twice a week unless the patient is only having weekly dressing changes. However, the patient's overall skin condition should be examined each shift. All findings must be recorded in the nursing notes with wounds or skin damage recorded on the body map in areas where this is used.

5.4.3. Wound documentation will include wound dimensions, using ruler measurements. This can be supported by body mapping and digital images. A wound assessment chart must be completed at each dressing change, except in cases where the dressing is changed multiple times a day. (See also the Wound Care Policy.)

5.4.4. The ward/unit nurse responsible for the patient must ensure a plan of care is provided, detailing measures to prevent further damage and assist with the treatment. This plan will include, for example, use of self-care strategies, pressure relieving equipment, positioning schedules, skin care requirements, moving and handling needs, nutritional and wound care requirements. This will usually be recorded on the aSSKINg care bundle. Specialist advice on treatments to be sought as required.

5.4.5. Nutrition and hydration is key in healing pressure ulcers. If food intake remains low refer to a dietitian early.

5.5 Patient and Carer participation

5.4.1 Staff will actively encourage patients/carers to participate in the prevention and treatment of those at risk of pressure damage. This will encompass strategies of repositioning, skin observation, safe use of the equipment and passive exercises.

5.4.2 Staff will utilise a range of information resources to assist with this process, for example, tissue viability resources or link nurses. All interventions and education must be clearly documented in the nursing care record.

5.4.3 The carer's policy should be referred to in all instances when pressure ulcer prevention or treatment is delegated to a carer. That care remains the responsibility of the registered practitioner, as does the communication and support of that carer in their caring role on the ward. All agreed on care to be delivered by the carer must be documented and evaluated in the patient's care plan in line with the policy.

5.5 Non-Concordance

5.5.1 Each time a patient declines to be repositioned it must be clearly documented in the nursing notes. Patients must continue to be offered assistance with repositioning at regular intervals, in accordance with their risk assessment.

5.5.2 A patient who declines to reposition more than 2 times in succession should be reviewed by the ward manager or their delegated deputy.

5.5.3 Reasons for non-concordance must be addressed and dealt with if possible i.e. pain when moving - patients may need analgesia to facilitate regular repositioning.

5.5.4 The plan of care should also be discussed with the patient, their family and or carers, including an explanation of the risk and potential outcomes, with clear documentation in the patient's clinical record that this has been done and that the patient, their family and carers where applicable understand those risks and potential outcomes.

5.5.5 A referral to tissue viability should be made at an early stage for any patient who is non-concordant with their repositioning care plan.

5.6 End of life care

5.6.1 For patients in the Last Days of Life, every effort should be made to continue to undertake regular total skin assessment and full implementation of the

aSSKINg care bundle as part of an individualised care plan and taking into account the wishes of the patient, their family and carers as applicable.

- 5.6.2 The PURPOSE T risk assessment, body map and aSSKINg care bundle should remain in the nursing notes for the duration of the patient's hospital stay at UHP.

5.7 Education

- 5.7.1 The Tissue Viability team support clinical educators to provide update sessions in pressure ulcer assessment and treatment.
- 5.7.2 The tissue viability team will endeavour to provide occasional additional education to teams in a variety of formats, as clinical priorities allow.
- 5.7.3 Staff involved in caring for patients at risk of developing, or who have developed a pressure ulcer have a professional responsibility to ensure that they attend training in pressure ulcer prevention and treatment.
- 5.7.4 A designated registered nurse, unregistered nurse or healthcare assistant will act as ward links and attend the tissue viability link meetings. Frequent non-attendance and failure to engage will be discussed with the ward/department manager.
- 5.7.5 All clinical staff are to complete induction pressure ulcer prevention training including the PURPOSE T & aSSKINg bundle UHP teaching video.

5.8 Safeguarding

- 5.8.1 The guidance issued by the Department of Health and Social Care (2024) in the document *Safeguarding Adults Protocol: Pressure Ulcers and the raising a safeguarding concern* should be followed.
- 5.8.2 Where there is concern that pressure ulceration has occurred, the practitioner should, in discussion with the individual and family, refer the individual to the appropriate local healthcare services, unless they are already in receipt of such services, even where they are in receipt of social care services. If a concern is raised that a person has severe damage, you should:

- complete the adult safeguarding decision guide [Pressure ulcers: how to safeguard adults - GOV.UK \(www.gov.uk\)](https://www.gov.uk/guidance/pressure-ulcers-how-to-safeguard-adults) (a shortened version is available on Datix).
- raise an incident immediately as per organisation policy

Note: severe damage in the case of pressure ulcers may be indicated in some cases by multiple category 2 or single category 3 or 4 ulcers, but could also be indicated by the impact the pressure damage has on the person affected (for example, pain) (DOH 2024).

- 5.8.3 The adult safeguarding support tool within this document should be used to aid any decision about whether to raise a safeguarding adults concern with the local authority.
- 5.8.4 For Safeguarding Children, any concerns with pressure damage or

MASD/IAD/NAD should be reported to the UHP Safeguarding Team via an internal SeeEHR form. If there are concerns of neglect, a children's social care referral is required to be submitted to the MASH or MARU. Details and referral forms are available on SharePoint. Any advice or guidance needed please contact the Safeguarding Children Team in office hours (4)39053.

- 5.8.5 For all new pressure ulcers, the Statutory Duty of Candour policy needs to be followed; refer to incident management policy. Category 3 & 4, Suspected deep tissue injuries (DTIs) and Unstageable minimum category 3 pressure ulcers will all require Duty of Candour.

5.9 Discharging patients with pressure ulcers or other skin damage

- 5.9.1 A patient who is deemed 'at risk' of developing a pressure ulcer or other skin damage should have a total skin assessment on the day of discharge.
- 5.9.2 A patient with a pressure ulcer or other skin damage should be discharged with a documented wound care assessment and plan and this will be recorded in the patient's notes. A digital image should be taken of the wound, using SNAP SeeEHR on the day of discharge unless contraindicated (e.g. specialist dressing in place).
- 5.9.3 A patient discharged with a pressure ulcer should be referred for further assessment by the community team(s). This referral should be documented in the patient's notes. Handover must be given to the ongoing provider if the patient is being discharged to a nursing/care home or community hospital.
- 5.9.4 The Ward/Department Manager will follow up patients with hospital acquired pressure ulcers that are unstageable – minimum category 3 or suspected DTI following discharge or transfer.

6 Overall Responsibility for the Document

- 6.1. The Tissue Viability Lead Nurse has overall responsibility for developing this policy in consultation with other relevant clinical staff.
- 6.2. Responsibility for implementation of this policy lies with clinical teams within Care Groups.
- 6.3. This policy will be reviewed in line with Trust Policy of five years and earlier if any significant changes are indicated.

7 Consultation and Ratification

- 7.1. The design and process of review and revision of this policy will comply with The Development and Management of Trust Wide Documents.
- 7.2. The review period for this document is set as a default of five years from the date it was last ratified, or earlier if developments within or external to the Trust indicate the need for a significant revision to the procedures described.
- 7.3. This document will be approved by the Harm Free group and ratified by the Chief Nurse.
- 7.4. Non-significant amendments to this document may be made, under delegated authority from the Chief Nurse, by the nominated author. These must be ratified by

the Chief Nurse and should be reported, retrospectively, to the approving board.

- 7.5. Significant reviews and revisions to this document will include a consultation with named groups across the Trust. For non-significant amendments, informal consultation will be restricted to named groups, or categories that are directly affected by the proposed changes.

8 Dissemination and Implementation

- 8.1. Following approval and ratification, this policy will be published in the Trust's formal documents library on Sharepoint and all staff will be notified through the Trust's normal notification process.
- 8.2. Document control arrangements will be in accordance with The Development and Management of Trust Wide Documents.
- 8.3. The document author(s) will be responsible for agreeing on the training requirements associated with the newly ratified document with the Chief Nurse and for working with the Trust's training function, if required, to arrange for the required training to be delivered.

9 Monitoring Compliance and Effectiveness

- 9.1. Incident Reporting of New Pressure Ulcers (Hospital Acquired) category 2-4, unstageable – minimum cat 3 and suspected DTIs as well as MASD. This information is available for Ward Managers / Matrons to review and trends will be reported back to Ward areas via the Matron Safety Huddle on a monthly and Harm Free Care Group on a bi-monthly basis by the Tissue Viability team.
- 9.2. All hospital acquired pressure ulcers of category 3 and 4 will have an Immediate Safety Review, as outlined in the Patient Safety Incident Response Plan and the Trust policy for investigation and reporting will be followed. Statutory Duty of Candour applies. Findings, outcomes and required actions will inform ongoing improvement efforts by the Pressure Ulcer Steering Group. Learning points and Action Plans will be reviewed at the Harm Free Care Group and disseminated across the Trust.
- 9.3. Process monitoring will be undertaken by Ward Managers/Matrons as part of the regular audits using Meridian/IQVIA. Ward Managers/Matrons will be responsible for implementing any remedial actions required, with support from the tissue viability team as needed.
- 9.4. The Trust will meet any statutory requirements for data collection/audit to meet external requirements (e.g. CQUINS) and action plans agreed at ward level dependent on results.

- European Pressure Ulcer Advisory Panel and National Pressure Ulcer Advisory Panel (2019) *Pressure Ulcer Guidelines*. Washington DC: National Pressure Ulcer Advisory Panel
- National Institute of Clinical Excellence, NICE (NG19) March 2015 Diabetic foot problems: prevention and management [Overview | Diabetic foot problems: prevention and management | Guidance | NICE](#)
- Pressure ulcers: prevention and management **(NICE)** Clinical guideline [CG179] Published April 2014 [Overview | Pressure ulcers: prevention and management | Guidance | NICE](#)
- National Wound Care Strategy Programme (NWCS) (2023) Pressure Ulcer Recommendations and Clinical Pathway. Health Innovation Network.
- Department of Health and Social Care (2024): Safeguarding adults protocol: pressure ulcers and raising a safeguarding concern [Safeguarding adults protocol: pressure ulcers and raising a safeguarding concern - GOV.UK \(www.gov.uk\)](#)
- Gero Langer, Ching Shan Wan, Astrid Fink, Lukas Schwingshackl, Daniela Schoberer (2024) Nutritional interventions for preventing and treating pressure ulcers. Cochrane Database Syst Rev 12;2(2):CD003216

Dissemination Plan			
Document Title	Prevention and Management of Pressure Ulcers		
Date Finalised	November 2024		
Previous Documents			
Action to retrieve old copies			
Dissemination Plan			
Recipient(s)	When	How	Responsibility
All Trust staff			Information Governance Team
Tissue Viability Link Nurses		Email	Lead Nurse for Tissue Viability
Matrons/Ward Managers		Email	Lead Nurse for Tissue Viability

Review Checklist		
Title	Is the title clear and unambiguous?	Yes
	Is it clear whether the document is a policy, procedure, protocol, framework, APN or SOP?	Yes
	Does the style & format comply?	Yes
Rationale	Are reasons for development of the document stated?	Yes
Development Process	Is the method described in brief?	Yes
	Are people involved in the development identified?	Yes
	Has a reasonable attempt has been made to ensure relevant expertise has been used?	Yes
	Is there evidence of consultation with stakeholders and users?	Yes
Content	Is the objective of the document clear?	Yes
	Is the target population clear and unambiguous?	Yes
	Are the intended outcomes described?	Yes
	Are the statements clear and unambiguous?	Yes
Evidence Base	Is the type of evidence to support the document identified explicitly?	Yes
	Are key references cited and in full?	Yes
	Are supporting documents referenced?	Yes
Approval	Does the document identify which committee/group will review it?	Yes
	If appropriate have the joint Human Resources/staff side committee (or equivalent) approved the document?	Yes
	Does the document identify which Executive Director will ratify it?	Yes
Dissemination & Implementation	Is there an outline/plan to identify how this will be done?	Yes
	Does the plan include the necessary training/support to ensure compliance?	Yes
Document Control	Does the document identify where it will be held?	Yes
	Have archiving arrangements for superseded documents been addressed?	Yes
Monitoring Compliance & Effectiveness	Are there measurable standards or KPIs to support the monitoring of compliance with and effectiveness of the document?	Yes
	Is there a plan to review or audit compliance with the document?	Yes

Review Date	Is the review date identified?	Yes
	Is the frequency of review identified? If so is it acceptable?	Yes
Overall Responsibility	Is it clear who will be responsible for co-ordinating the dissemination, implementation and review of the document?	Yes

Core Information	
Date	November 2024
Title	Prevention and Management of Pressure Ulcers Policy
What are the aims, objectives & projected outcomes?	To ensure compliance with the external guidance and recommendations for prevention and management of pressure ulcers. Implementation will reduce co-morbidity, enable patients to return their normal activity and home or place of care, reduce inpatient time and health care costs. The application will improve communication and ensure consistency of care across healthcare providers
Scope of the assessment	
Pressure ulcer prevention forms a key part of clinical care for patients in all areas of the Trust and all clinical staff are expected to take responsibility for ensuring all appropriate measures are taken to minimise risk.	
Collecting data	
Race	There is no evidence to suggest that there is an impact on race regarding this policy. Darker skin tones can make identification of skin damage more difficult. However, data collected from Datix incident reporting and complaints will ensure this is monitored. Consideration will be made if the information provided is required in a different language.
Religion	There is no evidence to suggest that there is an impact on religion or belief and non-belief regarding this policy. However, data collected from Datix incident reporting and complaints will ensure this is monitored. The plan of care for patients includes nutritional requirements which may relate to specific religious beliefs.
Disability	There is no evidence to suggest that there is an impact on disability regarding this policy. However, data collected from Datix incident reporting and complaints will ensure this is monitored. The document considers issues that may be identified in the plan of care for patients and recommendations will be made as required Consideration will be made if the information is required in different formats or the translation services are required. Consideration has been made for vulnerable adults and mental health and learning disability issues will be highlighted within the care plan as appropriate.

Sex	There is no evidence to suggest that there is an impact on sex regarding this policy. However, data collected from Datix incident reporting and complaints will ensure this is monitored.
Gender Identity	There is no evidence to suggest that there is an impact on gender identity regarding this policy. However, data collected from Datix incident reporting and complaints will ensure this is monitored.
Sexual Orientation	There is no evidence to suggest that there is an impact on sexual orientation regarding this policy. However, data collected from Datix incident reporting and complaints will ensure this is monitored.
Age	There is no evidence to suggest that there is an impact on age regarding this policy. However, data collected from Datix incident reporting and complaints will ensure this is monitored.
Socio-Economic	There is no evidence to suggest that there is an impact on socio-economic regarding this policy. However, data collected from Datix incident reporting and complaints will ensure this is monitored.
Human Rights	The document has considered safeguarding issues which will be addressed following the Trust safeguarding processes. Informal carers will be encouraged to participate in the prevention and treatment as required. Data collected from Datix incident reporting and complaints will ensure this is monitored.
What are the overall trends/patterns in the above data?	No comparative data have been used to date which means that no trends or patterns have been identified
Specific issues and data gaps that may need to be addressed through consultation or further research	No gaps have been identified at this stage, but this will be monitored via data collected from Datix incident reporting and complaints.

Involving and consulting stakeholders				
Internal involvement and consultation	Matrons			
	Ward Managers			
External involvement and consultation	Specialist Staff – safeguarding, vascular, diabetic podiatry			
	Tissue viability & link nurses			
Chief Nurse				
Impact Assessment				
Overall assessment and analysis of the evidence	Consideration will be made if the information provided is required in a different language.			
	The plan of care for patients includes nutritional requirements which may relate to specific religious beliefs.			
	The document considers issues that may be identified in the plan of care for patients and recommendations will be made as required			
	Consideration will be made if the information is required in different formats, or the translation services are required.			
	Consideration has been made for vulnerable adults and mental health and learning disability issues will be highlighted within the care plan as appropriate.			
	The document has considered safeguarding issues which will be addressed following the Trust safeguarding processes.			
Informal carers will be encouraged to participate in the prevention and treatment as required.				
Action Plan				
Action	Owner	Risks	Completion Date	Progress update